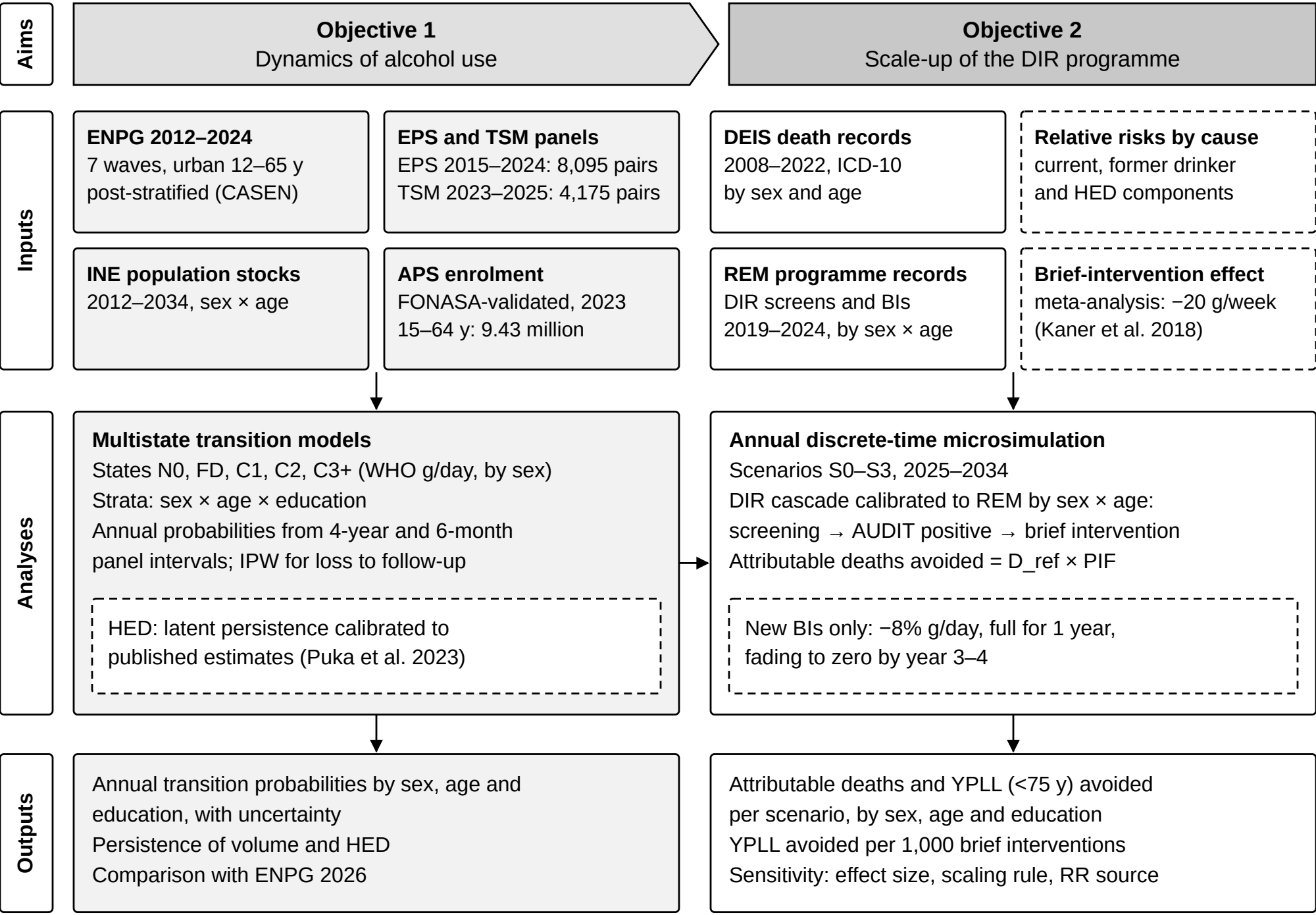
Aims
Objective 1
Dynamics of alcohol use
Objective 2
Scale-up of the DIR programme
Inputs
ENPG 2012–2024
7 waves, urban 12–65 y
post-stratified (CASEN)
EPS and TSM panels
EPS 2015–2024: 8,095 pairs
TSM 2023–2025: 4,175 pairs
DEIS death records
2008–2022, ICD-10
by sex and age
Relative risks by cause
current, former drinker
and HED components
INE population stocks
2012–2034, sex × age
APS enrolment
FONASA-validated, 2023
15–64 y: 9.43 million
REM programme records
DIR screens and BIs
2019–2024, by sex × age
Brief-intervention effect
meta-analysis: −20 g/week
(Kaner et al. 2018)
Analyses
Multistate transition models
States N0, FD, C1, C2, C3+ (WHO g/day, by sex)
Strata: sex × age × education
Annual probabilities from 4-year and 6-month
panel intervals; IPW for loss to follow-up
HED: latent persistence calibrated to
published estimates (Puka et al. 2023)
Annual discrete-time microsimulation
Scenarios S0–S3, 2025–2034
DIR cascade calibrated to REM by sex × age:
screening → AUDIT positive → brief intervention
Attributable deaths avoided = D_ref × PIF
New BIs only: −8% g/day, full for 1 year,
fading to zero by year 3–4
Outputs
Annual transition probabilities by sex, age and
education, with uncertainty
Persistence of volume and HED
Comparison with ENPG 2026
Attributable deaths and YPLL (<75 y) avoided
per scenario, by sex, age and education
YPLL avoided per 1,000 brief interventions
Sensitivity: effect size, scaling rule, RR source
Estimated or calibrated with Chilean data
External parameter or modelling assumption